Triage Assessment Form: Crisis Intervention (Revised)

Client Name: _____ Time/Date: _____

Crisis Worker: _____ Contact Type: _____ Phone: _____ Office ____ Field

Crisis Event: _____

_____

_____

Disposition _____

_____

_____

Observations (Check as many that apply)

__ off medication
__ medication not effective [a]
__ hallucinating [a] (__ smells __sights __sounds __ touch)
___ bizarre behavior/ appearance
___ poor hygiene
___ absurd, illogical speech [a]
___ paranoid/suspicious thoughts [a]
___ flashbacks, loss of reality contact
___ under the influence of mood altering substance
___ other (explain)

___ oppositional defiant to verbal suggestions
___ aggressive gestures [b]
___ reckless behavior
___ self injurious behavior
___ physically violent [b]
___ verbal threats to self or others
___ suicidal/homicidal thinking/verbalizing
___ suicidal/homicidal gestures/behaviors [b]
___ suicidal/homicidal plan clear [b]

___ uncooperative
___ flat affect
___ impulsivity
___ hysterical
___ confusion
___ unable to follow simple directions
___ unable to control emotions
___ cannot recall personal information (phone, address)
___ situation perceived as unreal (spectator)
___ nonresponsive [a]

Notes: _____

_____

[a] psychiatric evaluation recommended ***[b] hold for law enforcement officers or EMTs***

Triage Assessment (X = Initial Assessment/ O = Terminal Assessment)

Affective

__ Anger __ Fear __ Sadness

1 2 3 4 5 6 7 8 9 10

Behavioral

__ Approach __ Avoidance __ Immobile

1 2 3 4 5 6 7 8 9 10

Cognitive

__ Transgression __ Threat __ Loss

1 2 3 4 5 6 7 8 9 10

___ Physical ___ Psychological ___ Relationship ___ Moral/Spiritual

Initial Total Score: _____ Terminal Total Score: _____ (if used)

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SEVERITY SCALES
Check those that apply

	1	2	3	4	5	6	7	8	9	10
	No Impairment	**Minimal Impairment**		**Low Impairment**		**Moderate Impairment**		**Marked Impairment**		**Severe Impairment**
AFFECTIVE	○ Feelings are appropriate.	○ Affect appropriate to situation.		○ Affect appropriate to situation.		○ Affect may be incongruent with situation.		○ Affect obviously incongruent with situation.		○ Hysteria or flat affect resulting in decompensation or depersonalization.
	○ Stable mood.	○ Brief periods during which mood is experienced slightly more intensively than situation warrants.		○ Mood is experienced slightly more intensely than situation warrants.		○ Mood is experienced noticeably more intensely than situation warrants.		○ Mood dictated by situation and pervades all areas of life.		○ Mood presents a danger to self and/others.
	○ Responses to questions/requests are calm and composed.	○ Responses to questions/requests are emotional but composed.		○ Responses to questions/requests vary from rapid and agitated to slow and subdued.		○ Responses to questions/requests are emotionally volatile or beginning to shut down.		○ Responses to questions/requests noncompliant due to interference of emotions.		○ Cannot respond to questions/requests because of interference of emotions.
BEHAVIORS	○ Coping behaviors appropriate to crisis event.	○ Coping behaviors mostly effective but not organized.		○ Occasional use of ineffective coping behaviors.		○ Coping behaviors may be ineffective and maladaptive.		○ Coping behaviors are likely to intensify crisis situation.		○ Coping behaviors are totally ineffective and accelerate the crisis.
	○ Performing tasks necessary for daily functioning unimpeded.	○ Performing tasks necessary for daily functioning minimally affected.		○ Performing tasks necessary for daily functioning done with effort.		○ Performing tasks necessary for daily functioning is noticeably compromised.		○ Performing tasks necessary for daily functioning markedly absent.		○ Unable to perform even simple tasks necessary for daily functioning.
	○ Threat or danger nonexistent.	○ Behavior demonstrates frustration, but is nonthreatening.		○ Behaviors minimal threat to self or others.		○ Behavior is a potential threat to self or others.		○ Behaviors are increasingly impulsive, uncontrolled, and may be harmful to self and others.		○ Behaviors are highly destructive possibly to cause injury/death to self or others.
	○ Behavior is stable and non-offensive.	○ Behaviors mostly stable and non-offensive.		○ Behavior becoming unstable and offensive.		○ Upon request, behaviors can be controlled with effort.		○ Behavior is very difficult to control even with repeated requests.		○ Behavior is erratic and unpredictable.
COGNITIVE	○ Concentration intact.	○ Client's thought may drift to crisis event but focus of thoughts is under volitional control.		○ Occasional disturbance of concentration.		○ Intrusive thoughts of crisis event with limited control with frequent disturbance of concentration		○ Client plagued by intrusiveness of thoughts regarding crisis event.		○ Gross inability to concentrate on anything except crisis event
	○ Client displays normal problem-solving and decision making abilities.	○ Problem-solving and decision making abilities minimally affected.		○ Client experiences recurrent difficulties with problem-solving and decision making abilities.		○ Problem-solving and decision making abilities affected by obsessiveness, self-doubt, and confusion.		○ Appropriateness of client's problem-solving and decision making adversely affected by obessiveness, self-doubt, and confusion.		○ Client so afflicted by obsessiveness, self-doubt, confusion that problem-solving and decision making skills have "shut down."
	○ Client's perception and interpretation match with reality of situation.	○ Client's perception and interpretation of event substantially matches reality.		○ Client's perception and interpretation of crisis event may differ in some aspects with reality of situation.		○ Client's perception and interpretation of crisis event may differ noticeably with reality of situation.		○ Client's perception and interpretation of crisis event may differ substantially with reality of situation.		○ Client's perception and interpretation of the event as not real and or reality of crisis denied.
	○ Thinking is predictable and intact.	○ Thinking is purposeful yet muddied by crisis event.		○ Thinking is puzzled and at times lacking purpose.		○ Thinking is confused and not focused.		○ Thinking is non-sequential, nonlinear, with increasing non-lucidity.		○ Reality so altered thinking appears to be psychotic.
	○ Decisions are considerate of others.	○ Decisions may not be considerate of others.		○ Decisions are inconsiderate of others.		○ Decisions are offensive and antagonistic of others		○ Decisions have the potential to be harmful to self or others.		○ Decisions are a clear and present danger to self and/or others.